

[DEMO-5] Turkana Maternal, Newborn & Child Health Strengthening Activity — monthly report

Project: [DEMO-5] Turkana Maternal, Newborn & Child Health Strengthening Activity
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Contents

- Executive Summary
- Progress Against the Work Plan
- Gender Equality and Women's Empowerment
- Environmental Compliance
- Collaboration, Learning and Adaptation
- Implementation Challenges and Actions Taken
- Priorities for Next Month
- Branding and Marking Compliance

Executive Summary

In June 2026, the USAID-funded Turkana Maternal, Newborn and Child Health (MNCH) Strengthening Activity, implemented by GEC with the Lodwar Community Health Alliance (LOCHA) across 14 facilities in Turkana Central, Turkana South and Loima sub-counties, Kenya, produced 8 activity records and 16 verified indicator results. This document is made possible by the generous support of the American people through the United States Agency for International Development (USAID). The contents are the responsibility of GEC and do not necessarily reflect the views of USAID or the United States Government.

Delivery advanced on several fronts. Community health education reached 3400 people (2108 female, 1292 male) against a monthly plan that the field team judged on track, alongside 1700 children screened for acute malnutrition and 120 referred to treatment. The project trained 22 health workers (13 female, 9 male) in Emergency Obstetric and Newborn Care (EmONC), Integrated Management of Childhood Illness (IMCI) and essential newborn care, and delivered equipment kits to 4 more facilities, taking the installed total to 11 of 14. Some results remained below their monthly share of targets: 760 first antenatal care visits against a life-of-activity target of 4200, 350 skilled deliveries against 2100, and 560 children receiving DTP3/Penta3 against 3300. Kenya Health Information System (KHIS) on-time reporting reached 84 percent against a 90 percent target.

The main recorded challenges were solar refrigerator failures at two facilities and cattle-raid insecurity near Lorigum that suspended outreach for a week; vaccines were moved to the nearest working cold chain, both refrigerators were repaired within eight days, and outreach resumed with an escort arranged by the Sub-County Commander. In July, the project will continue training cohorts, mentorship visits, equipment delivery, outreach, screening and monthly data reviews.

Progress Against the Work Plan

Progress by Intermediate Result (IR), June 2026.

IR1: Health worker capacity and facility systems.

Project clinical trainers and nurse mentors trained 22 health workers (13 female, 9 male) in EmONC, IMCI and essential newborn care at Lokichar Health Centre on 9 June 2026 and completed 14 facility clinical mentorship visits across the 14 supported facilities. The logistics team delivered and installed essential MNCH equipment kits at 4 facilities on 14 June, with handover certificates signed by facility in-charges, taking the cumulative total to 11. A monthly data review and KHIS data quality audit at Lodwar on 25 June brought together 28 facility in-charges and records officers, and 84 percent of supported facilities submitted their KHIS monthly reports on time, up from 79.

IR2: Service delivery.

Supported facilities recorded 760 first antenatal care visits, 350 deliveries attended by skilled birth attendants and 34 women referred for emergency obstetric care using the transport voucher scheme, alongside 32 outreach immunization sessions that delivered DTP3/Penta3 to 560 children under one year (280 female, 280 male). ANC4+ coverage reached 40 percent (up from 36), skilled delivery coverage 54 percent (up from 50) and DTP3/Penta3 coverage 74 percent (up from 70). The project delivered these services with county health staff, facility teams and community health volunteers in Turkana County.

IR3: Community engagement and data systems.

The project trained and kitted 40 CHVs (26 female, 14 male) on the community health package, iCCM, MUAC screening and danger-sign counselling. CHVs and community mobilisers reached 3400 people (2108 female, 1292 male) with health education, screened 1700 children aged 6-59 months for acute malnutrition using MUAC (867 female, 833 male) and referred 120 children with acute malnutrition to treatment.

Indicator performance table

Indicator	Month result	Cumulative result	Life-of-activity target	Percent of target
ANC4+ coverage (HL-OC1a)	40 %	40 %	45 %	88.9 %
Skilled birth attendant coverage (HL-OC1b)	54 %	54 %	60 %	90 %
DTP3/Penta3 coverage under 1 year (HL-OC1c)	74 %	74 %	80 %	92.5 %
Health workers trained in EmONC, IMCI or essential newborn care (HL-1.1a)	22	86	120	18.3 %
Facility clinical mentorship visits (HL-1.1b)	14	50	84	16.7 %
Facilities equipped with essential MNCH kit (HL-1.2a)	4	11	14	28.6 %
First ANC visits (HL-1.3a)	760	2500	4200	18.1 %
Skilled deliveries (HL-1.3b)	350	1120	2100	16.7 %
Emergency obstetric care transport voucher referrals (HL-1.3c)	34	96	180	18.9 %
Children under 1 year receiving DTP3/Penta3 (HL-1.4a)	560	1790	3300	17.0 %
Outreach immunization sessions (HL-1.4b)	32	108	180	17.8 %
Facilities submitting KHIS reports on time (HL-1.5a)	84 %	84 %	90 %	93.3 %
CHVs trained and kitted (HL-2.1a)	40	190	220	18.2 %
People reached with community health education (HL-2.2a)	3400	10900	18000	18.9 %
Children 6-59 months screened with MUAC (HL-2.3a)	1700	5450	9000	18.9 %
Children with acute malnutrition referred to treatment (HL-2.3b)	120	380	650	18.5 %

Indicators behind plan and corrective action.

All indicators remain below their life-of-activity targets. Immunization numbers dipped slightly against plan because solar refrigerators failed at two facilities and cattle-raid insecurity near Loruugum suspended outreach there for a week. The project moved the vaccines temporarily to the nearest working cold chain, repaired both refrigerators within eight days, and resumed outreach with an escort arranged by the Sub-County Commander. The project recorded that a weekly cold-chain temperature check shared by phone with the sub-county cold-chain officer catches failures before vaccines are lost, and it will continue monthly cold-chain checks and follow up on late-reporting facilities through the monthly KHIS data review.

Rationale.

The June plan prioritised continued training, mentorship, equipment delivery, service delivery and community-level prevention, and mentorship visits and community education remained on plan. Equipment kits reached four more facilities, taking the total to eleven, and skilled delivery rose to more than half of deliveries at supported facilities. KHIS reporting became timelier. Field report, June 2026 (LOCHA programme team).

Planned activities for the next reporting period.

The project will continue training cohorts of health workers and hold monthly mentorship visits, deliver the next batch of essential MNCH equipment kits to the remaining facilities, continue outreach ANC clinics and voucher reimbursements to transporters, continue outreach immunization sessions and monthly cold-chain checks, train the next CHV cohort and supervise those already trained, continue dialogue days and radio messages, continue monthly MUAC screening and track referral completion, and follow up on late-reporting facilities through the monthly KHIS data review.

Gender Equality and Women's Empowerment

Sex-disaggregated results were recorded for the June people-level indicators and activities. The project trained 22 health workers (13 female, 9 male) in EmONC, IMCI and essential newborn care. It trained and kitted 40 CHVs (26 female, 14 male). Community health education reached 3400 people (2108 female, 1292 male). Immunization reached 560 children under one year (280 female, 280 male). MUAC screening covered 1700 children aged 6 to 59 months (867 female, 833 male). First antenatal visits totalled 760 women (760 female, 0 male).

Actions this month addressed gender gaps in access to services. Women attendants accompanied the outreach teams for deliveries and family planning counselling. The transport voucher scheme supported 34 women referred for emergency obstetric care in June, up from 30 in the previous period. Community dialogues were timed to market days and water-point visits, which the activity records note reached more men. The maternal death notification was reported through the MPDSR channel with no identifying details recorded.

No gender assessment or safeguarding compliance record was supplied for this month, so compliance cannot be stated from the records.

Environmental Compliance

The project remained compliant with the Initial Environmental Examination (IEE) mitigation measures in June 2026, and it recorded no environmental incidents or unexpected impacts in the month.

Health care waste management (sharps, placenta pits, incinerators).

The project completed waste management checks at all 14 supported facilities under the IEE in June. The project repaired two solar refrigerators that failed and returned the failed batteries to the supplier for recycling rather than discarding them. USAID marking was applied to the repaired refrigerators, to the four new equipment kits delivered in June, and the project replaced one health worker training banner after weather damage.

Any environmental incidents or unexpected impacts.

The project recorded no environmental incidents or unexpected impacts in June. The cold-chain failures at two facilities were equipment faults that the team corrected within the month, with vaccines temporarily moved to the nearest working cold chain and the refrigerators repaired within eight days.

The project required no remediation steps this month, as the records show no non-compliance with the IEE mitigation measures.

Collaboration, Learning and Adaptation

The project coordinated closely with county structures in June 2026 and adapted its outreach and cold-chain arrangements after two solar refrigerators failed and insecurity suspended outreach near Lorugum for a week.

Observation.

The team briefed the Sub-County Commander on the security situation near Lorugum and agreed an escort for outreach, and the county cold chain officer joined the repair planning for the failed refrigerators. The monthly data review and Kenya Health Information System (KHIS) data quality audit with sub-county health records officers covered all 14 facilities, and 84 percent of supported facilities submitted their monthly reports on time. The partner organization Lodwar Community Health Alliance (LOCHA) delivered the field activities. Activity records note that on-site mentorship after classroom training improves use of partographs and neonatal resuscitation, peer supervision by experienced community health volunteers (CHVs) keeps newly trained volunteers active, pre-agreed kit lists with the county pharmacist avoid duplicate supplies, dialogues timed to market days and water-point visits reach more men, fixed outreach calendars shared with CHVs increase turnout, screening during routine CHV household visits finds children who would otherwise be

On security, access, supply chain and workforce issues: the cold-chain failures, the one-week suspension of outreach near Lorugum, the Kitale road shipment delays, and the CHV training and data-entry constraints were the issues recorded for June. The records contain no request for USAID support for the month.

Priorities for Next Month

July priorities carry forward the June activity records and the lessons recorded this month. The project will continue training cohorts of health workers and hold monthly clinical mentorship visits, following the 22 health workers trained and 14 mentorship visits completed in June. It will deliver the next batch of essential MNCH equipment kits to the remaining facilities, after 4 facilities received and signed for kits in June, taking the total to 11 of the 14 targeted. The project will train the next community health volunteer (CHV) cohort and supervise those already trained, following 40 CHVs trained and kitted in June. Community dialogue days, household visits and radio messages will continue, as will outreach antenatal care (ANC) clinics and voucher reimbursements to transporters, which served 34 women in June. Immunization outreach sessions and monthly cold-chain checks will continue; the team will apply the June lesson that a weekly cold-chain temperature check shared by phone with the sub-county cold-chain officer catches failures before vaccines are lost, and outreach will proceed with the escort agreed with the Sub-County Commander. CHVs will continue monthly MUAC screening of children aged 6 to 59 months, and the project will track referral completion for the 120 children referred in June. The monthly data review and Kenya Health Information System (KHIS) data quality audit will continue, with follow-up on facilities that still enter data late after staff rotations. The team will apply the June lessons, including on-site mentorship after classroom training to support use of partographs and neonatal resuscitation, peer supervision by experienced CHVs, fixed outreach calendars shared with CHVs, and printing the reporting-rate dashboard for the review meeting.

Branding and Marking Compliance

The project met USAID branding and marking requirements in June 2026 and is compliant for the month. USAID marking was applied to the two repaired solar refrigerators and to the 4 essential maternal, newborn and child health (MNCH) equipment kits delivered in June, which brought the total to 11 of the 14 targeted facilities. One health worker training banner damaged by weather was replaced with a marked banner. No events or materials required remediation, and no corrective steps are needed.

Indicator Progress

HL-OC1a — Percentage of pregnant women with four or more ANC visits (ANC4+) at supported facilities (baseline 28, target 45, achievement 40 %, status VERIFIED)

HL-OC1b — Percentage of deliveries attended by a skilled birth attendant at supported facilities (baseline 41, target 60, achievement 54 %, status VERIFIED)

HL-OC1c — Percentage of children under 1 year who received DTP3/Penta3 at supported facilities (baseline 62, target 80, achievement 74 %, status VERIFIED)

HL-OC2a — Percentage of caregivers of children under five who can name at least three child danger signs (baseline 31, target 60, achievement 0 %, status DRAFT)

HL-1.1a — Number of health workers trained in EmONC, IMCI or essential newborn care (baseline 0, target 120, achievement 22 health workers, status VERIFIED)

HL-1.1b — Number of facility clinical mentorship visits conducted (baseline 0, target 84, achievement 14 visits, status VERIFIED)

HL-1.2a — Number of health facilities equipped with the essential MNCH equipment kit (baseline 0, target 14, achievement 4 facilities, status VERIFIED)

HL-1.3a — Number of pregnant women making a first ANC visit at supported facilities (baseline 0, target 4200, achievement 760 women, status VERIFIED)

HL-1.3b — Number of deliveries attended by a skilled birth attendant at supported facilities (baseline 0, target 2100, achievement 350 deliveries, status VERIFIED)

HL-1.3c — Number of women referred for emergency obstetric care using the transport voucher scheme (baseline 0, target 180, achievement 34 women, status VERIFIED)

